Final Equity Audit on BGSU Health Insurance

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The United States healthcare system is one of the most efficient, but complex and costly, compared to others. Unlike most countries offering universal healthcare, the United States requires individuals to select from multiple public, private, or employer-based insurance plans, often with high deductibles and complex cost-sharing mechanisms Paez, Mallery, Noel, Pugliese, McSorley, Lucado, & Ganachari, 2014). This situation is often new and intimidating for college students, especially international students. The introduction of health insurance on U.S. campuses came about in response to World War II, as schools started offering health plans to primarily domestic students who were no longer eligible for their families' insurance plans (McIntosh, Desai, Lewis, Jones, & Nunez-Smith, 2013). Growing recognition for students' health needs, combined with rising medical costs, were some main drivers for this shift. These plans initially served to make healthcare available to U.S. students who were moving past their parents' insurance plans or living at considerable distances from their homes.

With time, the demography of students widened to incorporate international students as they enrolled in universities across the country, which made them enroll in health insurance, citing legal liability, visa compliance, and campus health standards (McIntosh, Desai, Lewis, Jones, & Nunez-Smith, 2013). Today, most universities, including Bowling Green State University (BGSU), require international students to either enroll in a university-sponsored plan or provide evidence of equivalent coverage. Health insurance is a vital component for international students not just as a requirement for a good immigration status but also to ensure that they have access to medical care when they are sick and maintain their overall well-being through preventive services. However, research has shown that international students are the least likely to use healthcare services in the U.S., largely because of the cost involved and low knowledge about insurance policies (Hyun, Quinn, Madon, & Lustig, 2007; Adegboyega, Nkwonta, & Edward, 2020). Unlike domestic students, who may still be covered under their parents' health insurance, international students often enter the U.S.A. without any prior exposure to American insurance systems; therefore, they need to enroll in health insurance offered by the University. While health insurance

policies on college campuses were once designed with domestic students in mind, the landscape has changed. International students now make up over 5% of the total U.S. higher education population (Institute of International Education, 2019), and their distinct experiences call for a reassessment of whether existing systems truly serve them or simply include them on paper.

This is an equity audit paper on the compulsory health insurance policy for international students at Bowling Green State University (BGSU). While the policy aims to provide equal access to healthcare, it does so by automatically enrolling all international students into a university-sponsored insurance plan without giving them the option to opt out and explore alternative options or potentially more affordable coverage. This one-size-fits-all model, though administratively efficient, disproportionately impacts international students who may already be facing language barriers, financial strain, and cultural adjustment (Adegboyega et al, 2020). This paper aims to provide constructive criticism and propose realistic solutions while applying three guiding theoretical frameworks. Firstly, the Deficit Thinking Framework (Valencia, 2012), which explains how institutions often place responsibility on students rather than acknowledging structural barriers. Second is Maslow's Hierarchy of Needs, adopted from (Trivedi, 2019), which offers a lens for understanding how unmet health needs affect students' ability to engage and succeed in academic life, and finally, George Mwangi and Yao's equity-driven lens of internationalization (George Mwangi and Yao, 2020) highlights the need to move beyond treating international students as financial assets and toward embedding equity into institutional policy and practice. This paper draws on these frameworks and supporting literature to explore whether BGSU's current approach truly supports international students or merely includes them. I conclude with specific, actionable recommendations directed at university decision-makers.

Overview of the Case

At Bowling Green State University (BGSU), international students are automatically enrolled in the university-sponsored health insurance plan for students as soon as they register for classes. The health insurance policy for international students (BGSU, 2024) is framed as a safeguard to ensure students maintain continuous coverage, yet it leaves international students with no meaningful choice. They cannot enroll in a more affordable private plan or opt out even when they already have comparable coverage. While BGSU does offer a waiver process, it is rarely accessible in practice due to vague criteria, limited communication, and strict deadlines. In contrast, domestic students are free to stay on a family plan or shop for other coverage options. Another aspect of the issue pertains to the deduction of student fees, particularly regarding the health insurance component. The total tuition and fee framework includes insurance fees. Nonetheless, these insurance charges are charged last when partial payments are submitted. If a student has a debt of $2,000, with $1,000 designated for insurance, a payment of $1,000 will not be used to cover the insurance portion. Rather, it will serve to balance out other elements. The insurance fee will stay unpaid until the student can settle the full amount. As a result, students are left without their insurance card and cannot access healthcare services until their account is completely settled. This payment structure poses a considerable obstacle to access, particularly for international students dealing with financial limitations, and it postpones essential medical care.

From an equity perspective, this difference in policy enforcement creates an unequal and unjust system. As explained by McNair, Bensimon, and Malcom-Piqueux (2020), equity requires more than offering equal resources to all students. It demands institutional accountability and a critical understanding of how policies and practices reinforce structural disadvantages. Equity-minded institutions should examine how systemic inequities shape student outcomes and take responsibility for redesigning policies that fail to serve all students equitably. The lack of flexibility in BGSU's policy overlooks the diverse financial, cultural, and legal realities of international students, reinforcing a structure where their needs remain peripheral.

As Adegboyega et al. (2020) point out, international students often enter the U.S. with limited familiarity with American healthcare and insurance systems. By removing their right to choose, BGSU places them in a structurally disadvantaged position, compounding existing vulnerabilities related to immigration, finances, and social support. The automatic enrollment policy also imposes a significant financial burden. The annual cost of the university insurance plan exceeds $2,000. This is a heavy expense for international students who are legally restricted to working only 20 hours per week during the semester. Many rely solely on campus jobs and savings to manage tuition, housing, and living expenses. As Paez et al. (2014) note, health insurance in the U.S. is one of the most complex and expensive systems to navigate, especially for those unfamiliar with it. These rules require international students to shoulder this cost without equal flexibility, which deepens economic inequity.

This policy also results in low healthcare utilization, as many international students at BGSU avoid using the Falcon Health Center due to a lack of understanding about what is covered, fear of unexpected costs, or prior negative experiences with billing or unclear communication. (provide that this is true specifically at BGSU (either from conversations with peers, institutional reporting, or research). This is a significant claim and would benefit from evidentiary support. Research shows that international students often forgo care even when insured due to poor health insurance literacy, lack of culturally responsive services, and confusing cost structures (Adegboyega et al., 2020; Mackert, Mabry-Flynn, Champlin, & Beal, 2017). Ultimately, what BGSU presents as a protective health policy that functions as an exclusionary practice. It applies to a one-size-fits-all mandate to a diverse student body without regard for international students in specific legal, financial, and cultural realities. In a truly equitable system, international students would receive tailored support in selecting health plans, access to waiver processes that work, and education to navigate U.S. healthcare systems effectively. Health insurance access is not just coverage; it is choice, clarity, and fairness.

Literature Review

International students are vital contributors to the academic, cultural, and economic fabric of higher education institutions across the United States. Their presence enriches campuses and supports institutional missions of diversity and global engagement. As George Mwangi and Yao (2021) argue, international students bring global perspectives that challenge dominant U.S.-centric paradigms and help institutions fulfill their internationalization agendas in more equitable and reciprocal ways. Yet, despite their contributions, international students are often situated in institutional frameworks that prioritize revenue generation over holistic inclusion, leading to disparities in access, support, and representation. Research by Elisa Lenssen's (2010) on health insurance satisfaction among graduate students revealed varying student satisfaction based on three key factors, namely knowledge of plan details, perceived freedom in provider choice, and financial capacity.

One study that included 253 graduate students showed that individuals having an in-depth knowledge of their insurance plans, including the ability to choose their own healthcare providers, had higher levels of satisfaction (Lenssen, 2010). Conversely, individuals who were not adequately informed often experienced an overly complex and unapproachable insurance system, most often due to confusing documents and lack of training by their institutions to effectively navigate such systems. While this study was conducted among graduate students, it has implications that reach far beyond them to international students at both the graduate and undergraduate levels. This is because international students, regardless of their academic status, invariably face language barriers, different healthcare models, and orientation challenges within U.S. insurance systems. Therefore, the study highlights an important concern that having insurance is not in itself an automatic right to receive care, most notably in situations where students lack essential resources or institutional support to effectively maximize their insurance (Lenssen, 2010).

While the study focused specifically on graduate students, its findings are highly applicable to international students at both the graduate and undergraduate levels because international students,

regardless of their academic level, often face language barriers, unfamiliar healthcare systems, and minimal orientation to U.S. insurance structures. Thus, the study highlights a crucial point: possessing insurance does not guarantee meaningful access to care, particularly when students lack the resources or institutional support needed to effectively use that insurance (Lenssen, 2010).

The qualitative research by Adegboyega et al. (2020) on international students to understand their health insurance literacy highlighted a pervasive confusion about insurance terminology, such as co-pays, deductibles, and covered services. Students often lacked clear information about what services were included in their plans or how to use them effectively. Many expressed frustrations at the absence of targeted information during university orientation sessions and the cost involved with health care emerged as a central barrier to students in addition to the hidden fees or continuous misunderstanding of payment responsibilities. The studies underscore the importance of education on American health insurance and culturally accessible communication from institutions.

Furthermore, a quantitative study by Liu, Arledge, Cai, Tao, and Li (2023) on over 143 international students at a Midwestern U.S. university reported that over 50% of respondents do not understand basic insurance terms and 80% of the students failed to calculate their expected financial responsibility in hypothetical care scenarios, while 44% delayed or avoided care due to cost and confusion of billing components. These findings quantify the personal and academic risks posed by health insurance illiteracy, with long-term consequences for student well-being, retention, and academic success. Afoakwa-Acheampong's (2024) narrative study on international students in the southwestern U.S. presents an in-depth look into the day-to-day realities of students navigating American healthcare. The study revealed recurring themes such as cultural dissonance, language barriers, fear of hidden costs, lack of trust in providers, and reliance on informal networks. Importantly, students reported that healthcare-related stress directly affected their academic performance.

Theoretical Framework

This analysis uses an integrated theoretical framework grounded in Maslow's Hierarchy of Needs, Deficit Thinking Framework and George Mwangi and Yao's equity-driven theory of internationalization. These frameworks work together to reveal how a simple health insurance policy shapes students' academic, emotional, and social well-being. Each theory contributes to understanding how international students navigate a system that often excludes their voices and undermines their needs.

Maslow's Hierarchy of Needs

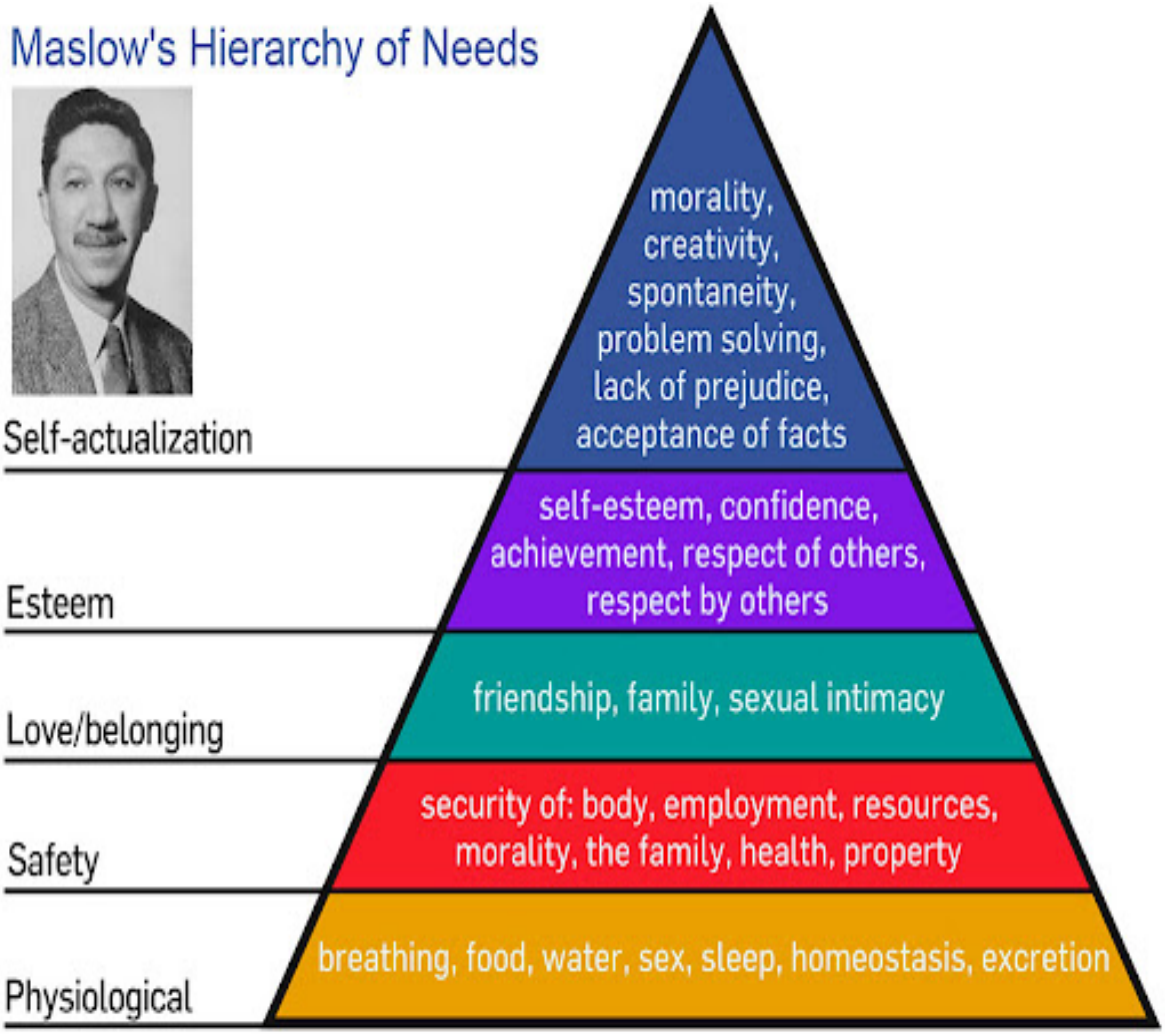

Figure 1

Maslow's Hierarchy of Needs. Adapted from *Maslow's Hierarchy of Needs – Theory of Human Motivation*, by A. J. Trivedi, 2019, *International Journal of Research in all Subjects in Multi Languages*, 7(6), p. 39.

Maslow's (1943) theory of needs, adopted from Trivedi (2019), is commonly used to explain how the needs of humans such as housing, food, security, and healthcare affect students in learning. The structure assumes that meeting the safety and physiological needs of students can help them achieve greater academic success. Maslow's theory has become significant in the success and overall well-being of students in achieving academic success (Crutchfield & Maguire, 2018; Goldrick-Rab, Clark, Baker-Smith, & Witherspoon, 2021). International students at BGSU cannot get an insurance card because they have not finished paying the fees in their student account and whenever they make any payments towards reducing the deficit in their student account, it is directed at paying other fees. Mostly, the fee allocated for health insurance is the last bill to be cleared.

In reality, international students cannot have access to medical care when they have not paid all the money in their student account. This is a major problem for international students and can hamper their health, leading to a deterioration and in the end, violating the important need in Maslow's hierarchy of needs. According to Liu (2023), over 80% of international students at a Midwestern university had financial difficulties due to health insurance fees and, as such, struggled to get medical care. This delayed their ability and reduced their chances of getting medical care. It is psychologically draining for international students to know that they are sick and unhealthy but cannot get access to medical care and have to rely on first aid from medicines they brought from home or from friends. Lie (2023) opines that this situation impedes academic focus, hampers integration, and threatens the sense of belonging of international students. It should be worth noting that Abraham Maslow's theory highlights equitability

not only in education but also in providing the necessary needs fairly to ensure academic success for students, in this scenario, international students.

Deficit Thinking Framework

According to Valencia (2019), there is stigmatization in higher education institutions because students do not excel academically, and this has been associated to the policies and systems in these institutions. The policies surround the racially minoritized, low-income, and mostly international students about how resources are disproportionately distributed to impede the progress and success of these populations of students within the institutions. The policy at BGSU makes international students begin their health insurance journey on a deficit note. Unlike domestic students, international students are enrolled in the health insurance provided by the university, and this comes with limited guidance, inflexible waiver options, and minimal orientation on how the U.S. health system works. When students struggle to access care or understand their benefits, the burden is placed on them to figure it out.

As Hyun, Quinn, Madon, and Lustig (2007) found, international students frequently experience confusion about co-pays, deductibles, and covered services, which leads to healthcare avoidance and delayed treatment. Rather than critically examining how institutional processes create these gaps in knowledge, BGSU's policy treats insurance coverage as an individual's responsibility. Valencia's (2012) framework calls attention to how such policies ignore the systemic failures—limited health literacy support, opaque billing systems, and rigid administrative procedures—that make it nearly impossible for international students to navigate care independently. In essence, deficit thinking shifts accountability away from the institution and onto the student, preserving inequity under the language of "uniform standards".

Equity-Driven Lens on Internationalization

George Mwangi and Yao (2021) offer a powerful critique of the current operation of internationalization in U.S. higher education. While internationalization is often promoted as a means of enhancing diversity and generating financial benefits for campuses, George Mwangi and Yao argue that institutions rarely assess whether their policies genuinely support the well-being of international students. Their equity-driven framework centers on power, justice, and the redistribution of resources, emphasizing that meaningful global engagement must address structural inequities rather than merely celebrate diversity. BGSU's policy on mandating health insurance enrollment without the option for waivers reflects a transactional approach to internationalization.

While the institution benefits international students' tuition and promotes their presence as a sign of diversity, it falls short of providing equitable support. The policy restricts students from selecting culturally appropriate or more affordable private insurance plans and offers little transparency or guidance on how to request exceptions or navigate the system. According to Reitmanova (2008), such exclusionary practices amount to "taxation without representation," where international students are expected to contribute economically without full rights or access. Mwangi and Yao (2021) urge institutions to critically deconstruct the assumption of neutrality in policymaking and to position international students as active participants in shaping policies that affect them. These equity-driven lenses call for reevaluation of decisions surrounding insurance access, perspectives of those considered and prioritized. This theoretical approach is particularly valuable to this equity audit because it shifts the focus from inclusive representation to inclusion as a form of power. It critiques performative diversity efforts and reframes internationalization as a political and ethical responsibility rather than a financial transaction.

Recommendation

Firstly, BGSU should implement mandatory health insurance education for all incoming and continuing international students as research by (Liu et al., 2023) has shown that over 50% of international students do not understand basic health insurance terms, and nearly 80% are unable to calculate their

financial responsibility in healthcare scenarios. This gap creates a delay in medical attention, increased anxiety, and unmet health need for international students which undermines the foundational levels of Maslow's (1943) Hierarchy of Needs. Particularly safety and physiological well-being, which are essential for individuals to thrive and pursue higher-order goals. To address this, BGSU's health insurance office, international programs and partnership should incorporate multilingual, interactive workshops into their virtual pre-arrival orientation and in-person programming upon arrival. These workshops should demystify the U.S. healthcare system to cover essential concepts and offer affordable insurance providers to enable student choice which of the providers they want. Incorporating these relatable case studies and translated visual aids, institutions can ensure that international students are not merely insured on paper but are meaningfully empowered to navigate and utilize their coverage with confidence.

Secondly, the university must simplify the structure and presentation of its insurance documents. Many students do not understand what their insurance covers until they encounter an emergency. This is because the documentation is overly technical and lacks visual clarity (Adegboyega et al., 2020). This confusion reinforces deficit-based narratives that place the burden of understanding on the student rather than on the institution (Valencia, 2019). To correct this, BGSU should translate all key documents into commonly spoken international languages and use infographics to illustrate key concepts. By doing so, it will elevate the language barrier and foster health literacy and shift responsibility back to the institution, in alignment with deficit-thinking critiques.

In case of the university's fee payment structure where BGSU processes health insurance charges last, meaning that international students who make partial payments toward their tuition may still be left without access to care until their account is fully settled. This process disproportionately affects international students, many of whom are allowed to work just a limited number of hours and who rely solely on on-campus employment to fulfill their financial needs (Paez et al., 2014). In terms of Maslow's hierarchy, the withdrawal of access to healthcare due to billing order undermines basic safety and security.

Therefore, the Bursar's Office and the Office of the Dean of Students should restructure payment sequencing to ensure insurance fees are prioritized for international students, or alternatively, allow temporary access to healthcare while the balance is being paid.

Moving forward, BGSU leaders must also advocate for policy reforms that give international students more flexibility in selecting insurance plans. George Mwangi and Yao (2021) caution that internationalization often functions as a financial strategy rather than an inclusive one. The current policy, which mandates enrollment in a single insurance plan with no waiver option, reflects a transactional model that overlooks the diversity of student needs. Institutional leaders including Office of the Dean of Students, International Programs and Partnerships (IPP), should engage in advocacy efforts at the state and national levels to allow for participation in public or subsidized plans, like policies adopted in Canadian provinces like Newfoundland and Labrador (Reitmanova, 2008).

Additionally, international students must have representation in the decision-making processes governing the structure of their healthcare. Their absence from advisory boards and policy planning bodies reinforces the perception that their experiences and input are not needed. Including international students on health services advisory boards would institutionalize shared accountability and help ensure that their voice is actively considered. Afoakwa-Acheampong's (2024) study documents the frustration many international students feel about being excluded from conversations that directly affect their well-being. Embedding their voices into university governance directly responds to George Mwangi and Yao's (2021) call to treat international students not as passive recipients but as policy co-constructors.

The International Programs and Partnerships (IPP) should create a plan for launching a peer health navigator project that can complement existing peer mentorship models. Peer programs help translate institutional systems into familiar, relatable terms, reducing confusion and increasing help-seeking behavior (Mackert et al., 2017). Upper-year international students should be trained to guide new arrivals in navigating the U.S. healthcare system. Peer navigators can offer one-on-one support at orientation and

ongoing support through the initial semester. This is based on both paradigm deficit and equity. The intervention leverages cumulative knowledge to address systemic deficits and to create a more welcoming and supportive environment for international students.

Lastly, Bowling Green State University (BGSU) should place emphasis on the recruitment of culturally competent and multilingual personnel in its health services department. Several students in Afoakwa-Acheampong's (2024) study reported experiencing stereotypes in their care interactions that discouraged them from returning for healthcare in the future. This finding agrees with Hyun et al. (2007), who argue that cultural misfits in care provision greatly contribute to avoidance and underuse. Hiring diverse personnel accompanied by training in cultural humility is essential in efforts to restore trust and in creating care settings where all students feel valued and heard.

Conclusion

This paper examined Bowling Green State University's health insurance policy for international students and concluded that, while seemingly offering ongoing protection, it inadvertently perpetuates limiting structural barriers that affect student health, agency, and academic performance. Through Maslow's Hierarchy of Needs, the Deficit Thinking Framework, and George Mwangi and Yao's equity-driven approach to internationalization, this analysis explains how this seemingly neutral policy marginalizes international students by restricting their choices, creating delays in receiving healthcare services, and existing in an inflexible, transactional manner.

In practice, the policy does not sufficiently recognize the various challenges faced by international students, including legal employment restrictions, cultural adaptation problems, language barriers, and lack of education on the U.S. healthcare system. Instead of creating structures that offer holistic support

for international students, the present system at BGSU is mainly geared towards efficiency and regulation, thus neglecting the target populations it claims to serve. The onus of navigating an expensive and complex healthcare system is often left to the students, while the university evades accountability for sustaining such faults.

This audit argues that inclusion is not enough unless it leads to equity in access, agency, and representation. Equity requires an all-out restructuring of support systems to shift from compliance to one that is intentional and student centric. The proposed measures, such as mandatory inclusive insurance education, fee structure alterations, peer navigator projects, and development of inclusive polices, are not abstract constructs. Instead, they represent evidence-based, theoretically grounded practical solutions that have the potential to reorient BGSU's internationalization from one that is centered for purposes of generating revenues to one that is care-focused, accountable, and just.

Overall, establishing equity in student affairs requires more than an acknowledgment of diversity but an ongoing institutional effort to deconstruct exclusionary systems and concurrently construct inclusive ones in cooperation with those students most impacted. Bowling Green State University has both the ability and obligation to lead such efforts to not just have international students represented on campus but to fully support them in their dignity, agency, and right to thrive. By embracing an equity-driven lens, BGSU can move from a framework of transactional policy implementation to that of a transformative model for the ideals of global engagement, shared responsibility, and educational equity.

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